

[PROVIDER-NAME]

[PROVIDER-NAME]

[PROVIDER-NAME]

Our Ref: (T) DRF/JN/ [MED-ID]

[DATE]

Clinic Letter : [DATE]

[PROVIDER-NAME]

[ADDRESS]

[ADDRESS]

[ADDRESS]

[POSTCODE]

Division of Unscheduled Care

Care Group – Medicine & Elderly Care Gastroenterology

F Level Centre Block CF91, Mailpoint [ADDRESS]

[ADDRESS]

[POSTCODE]

Telephone: [PHONE]

Fax: [PHONE]

Dear [PROVIDER-NAME] [NAME] , [ADDRESS]. [POSTCODE]

DOB: [DATE] , Hospital Number: [MED-ID] , NHS Number: [NHS-NO]

I reviewed this lady today. Her colonoscopy showed a total colitis and [NAME] biopsies show chronic inflammatory bowel disease without specific features. [NAME] appearances at endoscopy those of a Crohn's colitis where as on histology they would be consistent with ulcerative colitis. I think [NAME] best categorisation for [NAME] time being would be indeterminate colitis. [NAME] of a lot of abdominal pain. It is rare for this to be a feature of inflammatory colitis without bowel involvement. I suspect that her pain relates as much to her multiple sclerosis as to her Crohn's and should be treated with a pain-modifying agent such as amitriptyline.

With regard to [NAME] Crohn's disease, unfortunately [NAME] prescription [NAME] was given was [NAME] and [NAME] should have been reducing her prednisolone by 5 milligrams weekly not daily. I have asked her to increase to 35 milligrams daily for a week and then reduce by 5 milligrams per week thereafter. I have given her my standard counselling for azathioprine and an azathioprine information leaflet and I have checked her TPMT level. We will review her in about three weeks' time with the aim of then starting her on azathioprine as long-term maintenance.

With kind regards

Yours sincerely

[PROVIDER-NAME]

Consultant Physician/Gastroenterologist

Honorary Senior Lecturer in Medicine

Regional Specialist Adviser for Gastroenterology

cc:

[NAME]

[NAME]

[NAME]